



METOCLOPRAMIDE TABLETS

COMPOSITION:

Each tablet contains
Metoclopramide HCL equivalent to
Anhydrous Metoclopramide HCL

10mg

PRESENTATION:

Light yellow, round, flat tablet, 7.1mm in diameter with a single score line on one side and marked KOTRA logo on the other side.

INDICATION:

Adult population:

- Axxel Metoclopramide Tablet is indicated in adults for :
 - Prevention of delayed chemotherapy induced nausea and vomiting (CINV).
 - Prevention of radiotherapy induced nausea and vomiting (RINV).
 - Symptomatic treatment of nausea and vomiting, including acute migraine induced nausea and vomiting.

Age 9-18 years patient:

Axxel Metoclopramide Tablet is indicated in patient (aged 9-18 years) for :
Prevention of delayed chemotherapy induced nausea and vomiting (CINV) as a second line option.

PHARMACOLOGY:

Mechanism of Action:

The action of metoclopramide is closely associated with parasympathetic nervous control of the upper gastro-intestinal tract where it has the effect of encouraging normal peristaltic action. This provides for a fundamental approach to the control of those conditions where disturbed gastrointestinal motility is a common underlying factor.

Pharmacokinetics:

Metoclopramide is metabolised in the liver and the predominant route of elimination of metoclopramide and its metabolites is via the kidney.

Renal impairment:

The clearance of metoclopramide is reduced by up to 70% in patients with severe renal impairment, while the plasma elimination half-life is increased (approximately 10 hours for a creatinine clearance of 10-50 mL/minute and 15 hours for a creatinine clearance <10 mL/minute).

Hepatic impairment:

In patients with cirrhosis of the liver, accumulation of metoclopramide has been observed, associated with a 50% reduction in plasma clearance.

DOSAGE AND ADMINISTRATION:

Total daily dose of metoclopramide, especially for children and young adults, should not normally exceed 0.5mg/kg body weight.

For oral administration only.

Adults:

The recommended single dose is 10mg, repeated up to three times daily. The maximum recommended daily dose is 30mg or 0.5mg/kg body weight. The maximum recommended treatment duration is 5 days.

Prevention of delayed chemotherapy induced nausea and vomiting (CINV) (patients aged 9-18 years):

The recommended dose is 0.1 to 0.15mg/kg body weight, repeated up to three times daily by oral route. The maximum dose in 24 hours is 0.5mg/kg body weight.

Dosing table :

Age	Body Weight	Dose	Frequency
9 - 18 years	30 - 60 kg	5mg	Up to 3 times daily
15 - 18 years	Over 60 kg	10mg	Up to 3 times daily

The maximum treatment duration is 5 days for prevention of delayed chemotherapy induced nausea and vomiting (CINV). Tablets are not suitable for use in children weighing less than 30kg. Other pharmaceutical forms / strengths may be more appropriate for administration to this population.

Frequency of administration:

A minimal interval of 6 hours between two administrations is to be respected, even if vomiting or rejection of the dose occurs.

Special population:

Elderly:

In elderly patients a dose reduction should be considered, based on renal and hepatic function and overall frailty.

Renal impairment:

In patients with end stage renal disease (Creatinine clearance ≤ 15mL/min), the daily dose should be reduced by 75%. In patients with moderate to severe renal impairment (Creatinine clearance 15-60mL/min), the dose should be reduced by 50%.

Hepatic impairment:

In patients with severe hepatic impairment, the dose should be reduced by 50%. Other pharmaceutical forms / strengths may be more appropriate for administration to these populations.

CONTRAINDICATIONS:

- Hypersensitivity to the active substance or to any of the excipients listed.
- Gastrointestinal haemorrhage, mechanical obstruction or gastrointestinal perforation for which the stimulation of gastrointestinal motility constitutes a risk.
- Confirmed or suspected pheochromocytoma, due to the risk of severe hypertension episodes.
- History of neuroleptic or metoclopramide-induced tardive dyskinesia.
- Epilepsy (increased crises frequency and intensity).
- Parkinson's disease.
- Combination with levodopa or dopaminergic agonists.
- Known history of methemoglobinemia with metoclopramide or of NADH cytochrome-b5 deficiency.
- Use in children less than 1 year of age due to an increased risk of extrapyramidal disorders.
- Metoclopramide should not be used during the first three to four days following operations such as pyloroplasty or gut anastomosis as vigorous muscular contractions may not help healing.

WARNING AND PRECAUTIONS:

- Avoid doses exceeding 0.5mg/kg/day.
- Extrapyramidal effects, especially dystonic reaction of metoclopramide are more likely to occur in children shortly after initiation of therapy, and usually with doses higher than 0.5mg per kg of body weight per day.

If vomiting persists the patient should be reassessed to exclude the possibility of an underlying disorder e.g. cerebral irritation.

Neurological Disorders: Extrapyramidal disorders may occur, particularly in children and young adults, and / or when high doses are used. These reactions generally occur at the beginning of the treatment, and can occur after a single dose. If extrapyramidal symptoms occur, metoclopramide should be discontinued immediately. These effects are generally completely reversible after treatment discontinuation; however, symptomatic treatment may be required (benzodiazepines in children, and / or anticholinergic antiparkinsonian medicinal products in adults). An interval of at least six hours should be respected between each dose even if vomiting or rejection of the dose occurs, in order to avoid overdose. Long term treatment with metoclopramide may cause potentially irreversible tardive dyskinesia, particularly in elderly subjects. Treatment should not exceed 3 months because of the risk of tardive dyskinesia. Treatment must be discontinued if clinical signs of tardive dyskinesia occur. Neuroleptic malignant syndrome has been described with metoclopramide in combination with neuroleptics and with metoclopramide monotherapy. Metoclopramide must be immediately discontinued if symptoms of neuroleptic malignant syndrome develop, and appropriate treatment should be initiated. Particular caution should be exercised in patients with underlying neurological disorders, and in patients receiving other centrally-acting drugs. Symptoms of Parkinson's disease may also be exacerbated by metoclopramide.

Kidney or Liver Failure: In patients with kidney failure or severe liver failure, a dose reduction is recommended. Metoclopramide may cause elevation of serum prolactin levels.

Methemoglobinemia: Methemoglobinemia, which could be related to NADH cytochrome b5 reductase deficiency, has been reported. If this occurs, treatment must be immediately and permanently discontinued, and appropriate measures initiated (such as treatment with methylene blue).

Cardiac Disorders: Serious cardiovascular undesirable effects, including cases of severe bradycardia, circulatory collapse, cardiac arrest and QT prolongation have been reported during administration of metoclopramide by injection, particularly via the intravenous route. Particular caution should be exercised when administering metoclopramide, particularly via the intravenous route, in elderly subjects, patients with cardiac conduction disorders (including QT prolongation), patients with uncorrected electrolyte imbalance, bradycardia and patients taking other drugs known to prolong QT interval.

Contains lactose: Patients with rare hereditary problems of galactose intolerance, total lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Care should be exercised when using Metoclopramide in patients with a history of atopy (including asthma) or porphyria. Metoclopramide should not be used in the immediate

post-operative period (up to 3-4 days) following pyloroplasty or gut anastomosis, as vigorous gastrointestinal contractions may adversely affect healing. Special care should be taken when administering Metoclopramide intravenously to patients with 'sick sinus syndrome' or other cardiac conduction disturbances. There have been very rare reports of abnormalities of cardiac conduction with intravenous metoclopramide. Metoclopramide should be used with care with other drugs affecting cardiac conduction.

Effects on ability to drive and use machines: Metoclopramide has moderate influence on the ability to drive and use machines. Metoclopramide may cause drowsiness, dizziness, dyskinesia and dystonia which could affect the vision and also interfere with the ability to drive and operate machinery.

Use in Pregnancy and Lactation:

Pregnancy:

A large amount of data on pregnant women (more than 1000 exposed outcomes) indicates no malformative toxicity nor foetotoxicity. Metoclopramide can be used during pregnancy if clinically needed. Due to pharmacological properties (as other neuroleptics), in case of metoclopramide administration at the end of pregnancy, extrapyramidal syndrome in newborn cannot be excluded. Metoclopramide should be avoided at the end of pregnancy. If metoclopramide is used, neonatal monitoring should be undertaken.

Breastfeeding:

Metoclopramide is excreted in breast milk at low level. Adverse reactions in the breast-fed baby cannot be excluded. Therefore metoclopramide is not recommended during breastfeeding. Discontinuation of metoclopramide in breastfeeding women should be considered.

DRUG INTERACTIONS:

Contra-indicated combination: Levodopa or dopaminergic agonists and metoclopramide have a mutual antagonism.

Combination to be avoided: Alcohol potentiates the sedative effect of metoclopramide.

Combination to be taken into account: Due to the prokinetic effect of metoclopramide, the absorption of certain drugs may be modified.

Anticholinergics and morphine derivatives: Anticholinergics and morphine derivatives may have both a mutual antagonism with metoclopramide on the digestive tract motility.

Central nervous system depressants (morphine derivatives, anxiolytics, sedative H1 antihistamines, sedative antidepressants, barbiturates, clonidine and related): Sedative effects of Central Nervous System depressants and metoclopramide are potentiated.

Neuroleptics: Metoclopramide may have an additive effect with other neuroleptics on the occurrence of extrapyramidal disorders.

Serotonergic drugs: The use of metoclopramide with serotonergic drugs such as SSRIs may increase the risk of serotonin syndrome.

Digoxin: Metoclopramide may decrease digoxin bioavailability. Careful monitoring of digoxin plasma concentration is required.

Cyclosporine: Metoclopramide increases cyclosporine bioavailability (C_{max} by 46% and exposure by 22%). Careful monitoring of cyclosporine plasma concentration is required. The clinical consequence is uncertain.

Mivacurium and suxamethonium: Metoclopramide injection may prolong the duration of neuromuscular block (through inhibition of plasma cholinesterase).

Strong CYP2D6 inhibitors: Metoclopramide exposure levels are increased when co-administered with strong CYP2D6 inhibitors such as fluoxetine and paroxetine. Although the clinical significance is uncertain, patients should be monitored for adverse reactions.

Metoclopramide may reduce plasma concentrations of atovaquone.

SIDE EFFECTS:

Adverse reactions that occurred are presented in the table below. Their frequency is defined using the following conventions: common; uncommon; rare; very rare, not known. Within each system organ class, adverse reactions are presented in order of decreasing seriousness.

System Organ Class	Frequency	Adverse Reactions
Blood and lymphatic system disorders	Not known	Methaemoglobinaemia, which could be related to NADH cytochrome b5 reductase deficiency, particularly in neonates; Sulfhaemoglobinaemia, mainly with concomitant administration of high doses of sulphur-releasing medicinal products

System Organ Class	Frequency	Adverse Reactions
Cardiac disorders	Uncommon	Bradycardia, particularly with intravenous formulation
	Not known	Cardiac arrest, occurring shortly after injectable use, and which can be subsequent to bradycardia; Atrioventricular block, Sinus arrest particularly with intravenous formulation; Electrocardiogram QT prolonged; Torsade de Pointes
Endocrine disorders*:	Uncommon	Amenorrhoea, Hyperprolactinaemia
	Rare	Galactorrhoea
	Not known	Gynaecomastia
Gastrointestinal disorders	Common	Diarrhoea
General disorders and administration site conditions	Common	Asthenia
Immune system disorders	Uncommon	Hypersensitivity
	Not known	Anaphylactic reaction (including anaphylactic shock particularly with intravenous formulation)
Nervous system disorders	Very common	Somnolence
	Common	Extrapyramidal disorders (particularly in children and young adults and / or when the recommended dose is exceeded, even following administration of a single dose of the drug), Parkinsonism, Akathisia
	Uncommon	Dystonia including oculogyric crisis, Dyskinesia, Depressed level of consciousness
	Rare	Convulsion especially in epileptic patients
Psychiatric disorders	Not known	Tardive dyskinesia which may be persistent, during or after prolonged treatment, particularly in elderly patients, Neuroleptic malignant
	Common	Depression
	Uncommon	Hallucination
Vascular disorder	Rare	Confusional state
	Common	Hypotension, particularly with intravenous formulation
	Not known	Shock, syncope after injectable use, Acute hypertension in patients with phaeochromocytoma, Transient increase in blood pressure

*Endocrine disorders during prolonged treatment in relation with hyperprolactinaemia (amenorrhoea, galactorrhoea, gynaecomastia).

The following reactions, sometimes associated, occur more frequently when high doses are used:

- Extrapyramidal symptoms: acute dystonia and dyskinesia, parkinsonian syndrome, akathisia, even following administration of a single dose of the medicinal product, particularly in children and young adults.
- Drowsiness, decreased level of consciousness, confusion and hallucination.

OVERDOSAGE & TREATMENT:

Symptoms: Extrapyramidal disorders, drowsiness, decreased level of consciousness, confusion, hallucination, and cardio-respiratory arrest may occur.

Management: In case of extrapyramidal symptoms related or not to overdose, the treatment is only symptomatic (benzodiazepines in children and / or anticholinergic anti-parkinsonian medicinal products in adults). A symptomatic treatment and a continuous monitoring of the cardiovascular and respiratory functions should be carried out according to clinical status.

STORAGE: Keep container well closed. Store below 30°C. Protect from light.

PACK QUANTITIES: Available in blister pack of 10 x 10's and 10 x 100's.

Further information can be obtained from doctor, pharmacist or the manufacturer.

Manufactured By and Product Registration Holder :

Kotra Pharma (M) Sdn. Bhd.
No.1, 2 & 3, Jalan TTC 12, (90082-V)
Cheng Industrial Estate,
75250 Melaka, Malaysia.



REGN 060623

MAL19921470AZ